

Describe common prosthetic devices and their care (artificial limbs, hearing aids, contact lenses, eyeglasses and dentures).

A. Types of prosthetic devices – cosmetic, adaptive and restorative

1. Artificial limbs; arms and legs

a. Specially fitted to the individual patient/resident

b. Nursing considerations

- 1) Ask charge nurse for guidelines
- 2) Observe patient's/resident's ability and/or assist participate in ADLs and ambulation
- 3) Assist patient/resident to apply brace as ordered
- 4) Maintain body alignment when in chair or bed
- 5) Keep call bell and personal items within reach
- 6) Assist patient/resident with ROM to affected muscles
- 7) Assist patient/resident with aids to foster ADL and independence
- 8) Use pad brace, if necessary
- 9) Use stump sock to reduce swelling
- 10) Give constant praise for rehabilitative efforts
- 11) Provide skin care at pressure points of device to prevent skin breakdown
- 12) Observe for complaint of arm/leg pain or numbness, or weakness with use of prosthetic device
- 13) Encourage physical strengthening exercises for involved and non-involved extremities

Describe common prosthetic devices and their care (artificial limbs, hearing aids, contact lenses, eyeglasses and dentures).

A. Types of prosthetic devices – cosmetic, adaptive and restorative

2. Contact lenses

- a. Cleanse contact lenses as directed
- b. Special caution due to fragile nature of contact lenses
- c. Store according to package directions
- d. Easily lost
- e. Report the following
 - 1) Redness or drainage from eyes
 - 2) Complaints of eye pain or blurred vision

3. Eyeglasses

- a. Clean daily or as needed
 - 1) Wash glass lenses with warm water, dry with soft tissue
 - 2) Use special cleansing solution tissue and cloths on plastic lenses
- b. Check for intact parts, such as temples and screws
- c. Encourage patient/resident to wear eyeglasses as needed
- d. Store glasses carefully to protect from breakage and damage
- e. Check food trays and bed linens for misplaced eyeglasses
- f. Label eyeglasses with patient's/resident's name and room number

Describe common prosthetic devices and their care (artificial limbs, hearing aids, contact lenses, eyeglasses and dentures).

A. Types of prosthetic devices – cosmetic, adaptive and restorative

4. Dentures

- a. Refer to Mouth Care Objective 6
- b. Label denture storage container with patient's/resident's name
- c. Label dentures with patient's/resident's identification
- d. Encourage patient/resident to wear dentures
- e. Examine dentures for rough surfaces, breaks or cracks
- f. Report findings to charge nurse
- g. Handle with care and clean thoroughly before storing
- h. Check food trays, bed linens, and wrapped up tissues or napkins for misplaced dentures

B. Prosthetic devices help maintain independent functioning and patient/resident self-esteem

List ways to maintain healthy skin and identify at risk patients.

A. Maintain skin health

1. Encourage well-balanced diet and fluids
2. Provide skin care
 - a. Bathe as necessary
 - b. Rinse off soap thoroughly
 - c. Apply lotion as necessary
 - d. Massage healthy skin
 - e. Keep skin clean and dry
3. Observe high-risk patients/residents for potential problems
4. Give special attention to bony prominences
 - a. Turn and position the patient/resident correctly
 - b. Keep pressure off of reddened or irritated areas
5. Do not massage red, purple, or irritated areas
6. Only massage healthy tissue around the area
7. Keep bed free of small objects and linen free of wrinkles
8. Keep patient/resident clean & dry
9. Prevent shearing and friction
10. Report and record observations

List ways to maintain healthy skin and identify at risk patients.

B. Factors that place patients/residents at high risk for skin breakdown

1. Mobility/sensory problems

a. Paraplegic or quadriplegic

b. CVA

c. Peripheral vascular disease

d. Bed rest or decreased mobility

e. Decreased sensation

f. COPD

1) Low oxygen level

2) Continuous high Fowlers

3) Medications

g. Circulatory problems

1) Diabetes

2) Arterial disease with decreased sensation (neuropathy)

3) Poor circulation and healing

List ways to maintain healthy skin and identify at risk patients.

2. Elimination

- a. Incontinent of stool or urine
- b. Diarrhea
- c. Diaphoresis
- d. Dehydrated
- e. Leaking tubes or drainage

3. Fluid status

- a. Edema (swelling)
- b. Dehydration

4. Nutrition/body structure

- a. Obese
- b. Thin
- c. Poor appetite
- d. Nutritional lab values are low
- e. Poor fluid balance
- f. Poor nail care

5. Other predisposing factors

- a. Medications like Prednisone make the skin fragile and heal slower
- b. Cancer
- c. Splints/casts/prosthetic devices
- d. Anemia
- e. Age



Describe causes, signs and symptoms, and staging of pressure sores.

A. Common conditions which can lead to pressure sores

1. Pressure leads to decreased blood flow and nutrition of an area resulting in tissue loss
2. Excessively wet or dry skin
3. Moving patients/residents causing shearing force

B. Stages; signs and symptoms

1. Stage 1

- a. Skin is not broken. The epidermis and dermis are intact
- b. Erythema (redness) that does not resolve within 15-30 minutes

2. Stage 2

- a. Skin not intact
- b. Partial skin loss of the inner and/or outer skin layer (epidermis is damaged and part of the dermis can be involved)
- c. Skin can be blistered, cracked, and open with erythema or appear as a shallow crater
- d. No necrotic or dead tissue is present
- e. Wound bed is moist, pink and painful

3. Stage 3

- a. Full-thickness skin loss with both epidermis and dermis gone
- b. Damage extends to, but not through the tissue that covers the muscle
- c. Can have part of dermis left with necrotic tissue
- d. Possible drainage

4. Stage 4

- a. Full skin loss with major destruction and tissue death
- b. Involves subcutaneous tissues – possible fatty tissue, muscle and/or bone
- c. Can see pink healthy cells, necrotic tissue and eschar
- d. Wound can tunnel or have undermining in skin surrounding wound
- e. Risk that bone may become infected (osteomyelitis)

Describe general nursing measures to prevent and treat pressure sores

A. Nursing measures to prevent and treat pressure sores

1. Mobility/sensory

- a. Control pressure by pressure-reducing devices or special beds
- b. Turn every two hours at least
- c. Position with proper support of the body and limbs
- d. Promote active range of motion exercises or assist patient/resident with them
- e. Teach or assist patient/resident using a wheel chair to change position frequently
- f. Prevent shearing and friction during lifting, moving, transferring and repositioning procedures
- g. Keep bed linens clean, crumb free, and avoid wrinkles
- h. Watch tubing (like Foley Catheter) does not cause pressure
- i. Remove patients/residents from toilets and bedpans promptly
- j. Encourage circulation by gentle massage around red area; not over red area
- k. Check skin every eight hours
- l. Keep head of bed (H.O.B.) at 30 degrees as much as possible to avoid sacral pressure



Describe general nursing measures to prevent and treat pressure sores

A. Nursing measures to prevent and treat pressure sores

2. Elimination

- a. Keep skin clean and dry; apply powder where skin touches skin. Watch diaphoretic (sweaty) patient/resident
- b. Check incontinent patient/resident every two hours and ensure dry and clean (even when using adult incontinence garment)
- c. Monitor incontinence garment– plastic edges near skin can cause irritation
- d. Avoid scrubbing or rubbing when bathing and drying
- e. Use blankets and pillows to prevent skin from being in contact with skin

3. Fluid status

- a. Elevate limb that has edema
- b. Monitor compression devices, such as anti-embolic stockings and ace bandages. Remove every eight hours, check skin and watch edges do not cut into skin
- c. Encourage fluids to 1500-2000ml/day or as per care plan

4. Nutrition status/body structure

- a. Encourage and assist with balanced diet
- b. Obese patient/resident; check skin folds
- c. Monitor bony prominences on thin patients/residents

5. Others; monitor casts, braces, and clothing items that may cause pressure against skin



Describe general nursing measures to prevent and treat pressure sores

B. Use of pressure-reducing devices

1. Nothing replaces basic nursing care
2. Still must turn position and keep patient/resident clean and dry
3. Types of pressure reducing devices
 - a. Bed cradle; keeps pressure off feet and lower legs
 - b. Sheepskin
 - c. Heel and elbow protectors – shearing effect
 - d. Egg crate type pads/gel cushions
 - e. Alternating pressure mattress
 - f. Air fluidization bed (for example Clinitron or KinAir)
 - g. Trochanter rolls
 - h. Flotation pads or cushions

C. Legal, Quality Assurance (QA) and Performance Improvement(PI) issues

1. Duty to keep patient/resident from harm and prevent sores
2. There are increased legal risks associated with patients/residents developing pressure sores when there are no preventive measures implemented
3. Utilization Review (UR) issues – huge increase cost to facility